the HMO solution. The present structure of incentives encourages doctors to earn super normal incomes by over-prescribing procedures and thus earning revenues and kickbacks. This is analogous to the emergence of the New Pension System in the context of money management. Left to itself, the private sector finds it hard to break away from the toxic sales practices of insurance companies and mutual funds, and the supernormal fees charged by fund managers who do not add value.

At the same time, system thinking is a highly difficult problem in public policy; at low levels of state capacity it is too easy for central planning to emerge with the wrong answers. A tremendous amount of work is now required, in policy thinking on healthcare in India, to find the right pathways for system design, while recognizing that system thinking is a difficult problem in public policy.

The correlation between GDP and health

In the international experience, when a country gets richer, the health of the population improves. This is a strong correlation.

However, like all correlations, we should not jump to the conclusion that there is a causal relationship between the two. In some respects, we can see how becoming richer enables better health in India. As an example, a richer family would have better nutrition, and be able to spend more on private healthcare. A richer family would generally have higher education, and better knowledge feeds into better health. However, there are also many elements in India, where increased prosperity has harmed the disease burden. As an example, we now have the new challenges of air quality and road safety, which are harming people today, which were not present a few decades ago.

We should not assume that higher GDP growth will always give us improvements in the health of the population, in line with what has happened on average in the international experience. That average international experience is predicated on sophisticated health policy